Driver Licence Medicals	ONEHEALTH GENERAL PRACTICE & URGENT CARE	
	IND:	UCC STD:

A Driver's Licence medical examination is required when applying for (or renewing) a heavy vehicle endorsement, when patient has a medical condition that may affect their ability to drive safely, or if patient is aged 75 years or older.

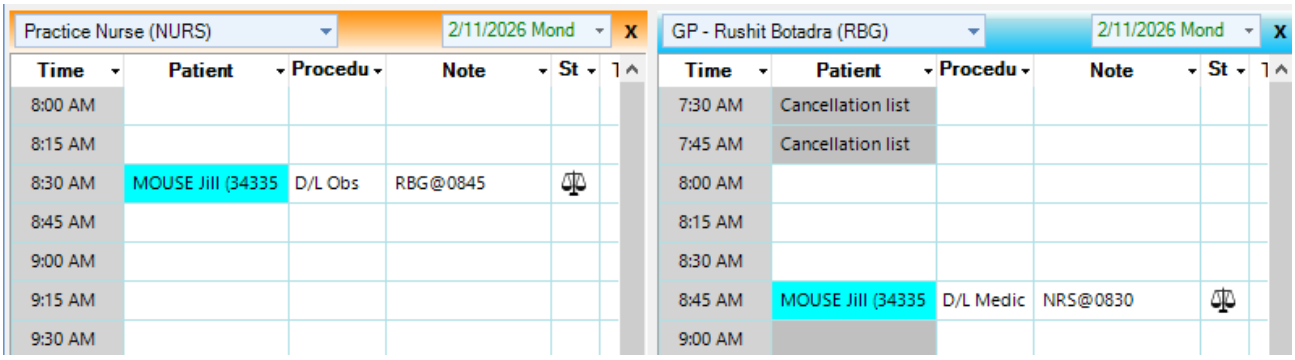
Driver Licence Medical services are available for **OneHealth enrolled patients only:**

- Under 75 years of age: 15 min GP appointment - normal GP fees apply.
- 75 years and over: 15min Nurse appointment, followed by 15min GP appointment. Fee: $100.00

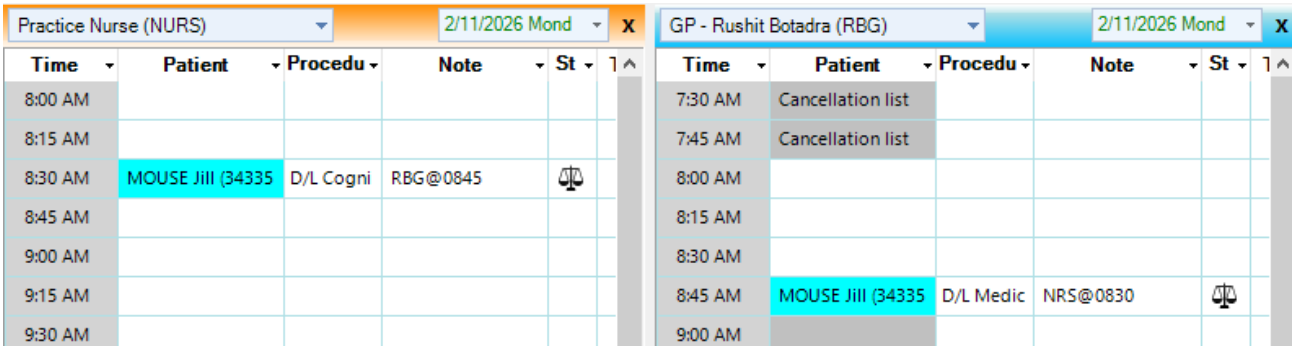
The above appointment times are for management of Driver Licence services only. Should a patient need repeat meds, or any other issue to be addressed within the appt, then FD to ensure that a 30-minute GP appointment is booked.

Receptionist uses the following Driver Licence procedure codes, ensuring appointments are linked:

- For PN Drivers Licence Appt: DLO – Driver Licence Observations
- For GP Drivers Licence Appt: DLM – Driver Licence Medical



Should GP observe a booked driver licence medical and pre-determines a cognitive test is required, then GP updates the PN appt procedure code from DLO to DLC (D/L Cognitive Test) and this will be completed prior to GP appointment:



If the Mini-Ace test score is < 21/30, then GP can proceed with the Trail Making Test. Alternatively, GP informs patient to re-book with nurse, so this test can be completed. (Nurse appt fee applies).

If no cognitive test is indicated for the nurse appointment, then nurse proceeds with basic obs including:

- Blood pressure, pulse
- Visual Acuity and assessment of Visual Fields.
- Check and update dashboard screening as required.

If during the GP appointment, a cognitive test is deemed necessary, then GP can proceed with the test if time permits. Alternatively, GP informs patient to rebook with nurse for a cognitive test (Nurse Consult fee applies).

If cognitive screening is appropriate, begin with the Mini-Ace as recommended by the RNZCGP. **Refer Schedule A**

NB: Non-enrolled patients, requesting D/L Medicals, are referred to Worldwise Travel.

SCHEDULE A

4. Changes to practice

Routine cognitive screening during fitness to drive assessments is no longer recommended. Use of cognitive screening tools remains at the health practitioner's discretion, including which tool is used.

- Cognitive screening should only be used to determine the presence or absence of cognitive impairment.
- This is likely to be relevant when there are concerns about cognitive function, or when the patient is unknown and there is insufficient information to judge cognitive capability.

If cognitive screening is appropriate, begin by screening for impairment:

- Mini-ACE is the recommended tool in most situations.
- For Māori patients aged 55 years and older, MANA uses the Hui process and includes an assessment of wairua and well-being, which should never be omitted.
- Where English is not the patient's first language, or where literacy is limited, RUDAS minimises the effect of cultural, educational, and linguistic diversity.

If screening indicates cognitive impairment, consider further testing relevant to driving performance. The Trail Making Test, particularly TMT-B, is the better indicator.

Broader assessment of cognitive function should occur and could include a traffic situational understanding and road safety test.

If driving safety remains uncertain, use practical driving assessment options:

- Referring the patient for an on-road safety test
  - Free, brief, and available nationally (e.g., VTNZ, AA, VINZ).
  - Provides a real-world assessment.
  - Considered as satisfactory evidence of on-road driving ability by NZTA.
- Referring the patient for an occupational therapy driving assessment
  - Assesses cognitive and functional aspects of driving.
  - Provides more specific information around cognitive ability.
  - Access is determined by cost and region.

5. Known cognitive impairment

In cases where it is already known that a patient has mild cognitive impairment, **do not use cognitive screening tools to determine fitness to drive**. Instead, as per NZTA recommendations and HealthPathways, comprehensive cognitive assessment is required, followed by on-road testing if appropriate.

People with dementia of moderate severity or worse are not medically fit to drive and do not require an occupational therapy driving assessment.